

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
**for the administration of yellow fever vaccine (Stamaril)
at designated Yellow Fever Vaccination Centres**

Guideline Summary: yellow fever vaccination

Designation is a precondition

- Yellow fever vaccine may only be given at a centre designated by NaTHNaC, which manages designation, training, registration, standards and audit for Yellow Fever Vaccination Centres in England, Wales and Northern Ireland. Scotland and the Crown Dependencies are managed separately.
- The vaccine may only be administered by a registered doctor, nurse, pharmacist or dentist working at the designated centre, with unrestricted registration and no pending investigation, warning, caution or suspension.
- This PGD must not be used at a pharmacy that does not hold current designation.

The vaccine

- Stamaril is a LIVE attenuated 17D vaccine. That is the single most important fact in this document: in an immunosuppressed patient the vaccine strain can replicate and cause extensive, severe and sometimes fatal infection.
- It is propagated in chick embryos, so egg allergy is relevant and anaphylaxis to egg is a contraindication here.
- A single 0.5 ml dose is given subcutaneously after reconstitution with the supplied solvent, and used immediately.

The certificate

- The International Certificate of Vaccination or Prophylaxis (ICVP) becomes valid 10 days after a first dose, so vaccination must be at least 10 days before travel where a certificate is required for entry.
- Since 11 July 2016 the certificate is valid for the life of the person vaccinated. It does not expire after 10 years.
- Where vaccination is contraindicated, travel is unavoidable and the destination requires a certificate, a Medical Letter of Exemption can be offered instead of vaccination.

Specialist advice

- NaTHNaC operates an advice line for health professionals on 020 7383 7474. Use it for any patient falling under the precautions below, and record the advice received.

Patient Group Direction

for the administration of

Yellow fever vaccine, live attenuated 17D (Stamaril)

for protection against yellow fever, at designated centres only

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must have completed immunisation training in line with the National Minimum Standards and Core Curriculum for Immunisation Training • All users must be competent in the recognition and management of anaphylaxis, and in vaccine handling and cold chain management • All users must previously have used PGD's to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005 • All users must be familiar with Gillick competence and the assessment of consent in children and young people, and with the involvement of a person with parental responsibility

Clinical condition or situation to which this PGD applies

PGD Indication	Active immunisation against yellow fever with Stamaril, live attenuated 17D vaccine, for individuals travelling to or residing in a country or area where yellow fever is a risk, or where an International Certificate of Vaccination or Prophylaxis is required for entry, in accordance with Chapter 35 of Immunisation Against Infectious Disease (the Green Book) and current NaTHNaC guidance. This PGD may only be used at a designated Yellow Fever Vaccination Centre.
Inclusion criteria	• Aged 9 months and over. • Travelling to or residing in an area where yellow fever vaccination is recommended by NaTHNaC, or where a certificate is required as a condition of entry, confirmed against TravelHealthPro at the time of the consultation. • Vaccination at least 10 days before travel where a certificate is required, so the certificate is valid in time. • Valid informed consent obtained, from a person with parental responsibility where the individual is under 16 and not Gillick

	competent. • Administered at a designated Yellow Fever Vaccination Centre by a practitioner authorised under that designation. • No contraindication as set out below.
Exclusion criteria	• Aged under 6 months. Absolute contraindication because of the increased risk of vaccine-associated encephalitis. • Aged 6 to 8 months. Outside this PGD: vaccination is only considered where the risk of transmission is high, such as during an outbreak, and travel is unavoidable. Seek specialist advice and refer. • Confirmed anaphylactic reaction to a previous dose of yellow fever vaccine. • Confirmed anaphylactic reaction to any component of the vaccine, or to egg. Egg allergy without anaphylaxis requires a detailed dietary history and may need specialist assessment; some individuals can be vaccinated in specialist centres. • History of a thymus disorder, including myasthenia gravis and thymoma, or thymectomy for any reason including during cardiac surgery. • A first-degree relative, meaning mother, father, full sibling or child, who has had yellow fever vaccine associated viscerotropic or neurologic disease (YEL-AVD or YEL-AND) not explained by a known medical risk factor. • Primary or acquired immunodeficiency, or immunosuppressive therapy. This includes leukaemia and lymphoma, severe immunosuppression due to HIV, cellular immune deficiencies, chronic lymphoproliferative disorders, stem cell transplant within 24 months, chemotherapy or radiotherapy within 6 months, solid organ transplant immunosuppression within 6 months, biological therapy within 12 months, and high dose corticosteroids or oral immune modulating drugs within 3 months at the thresholds set out in the NaTHNaC table. • Acute severe febrile illness. Postpone until recovered, allowing 10 days before travel for the certificate to become valid. • Any patient falling under the precautions below where specialist advice cannot be obtained before vaccination.
Cautions including any relevant action to be taken	• Facilities and trained staff for the management of anaphylaxis must be available, with immediate access to adrenaline (epinephrine) 1 in 1,000 injection and a telephone. • Aged 60 years and over: the risk of YEL-AND and YEL-AVD rises with age, and almost all cases occur with a first dose. Vaccinate only where there is significant and unavoidable risk after a detailed risk assessment. Do not vaccinate someone aged 60 or over who is only visiting areas where WHO considers the potential for exposure low, or where vaccination is generally not recommended. • Pregnancy: advise against travel to a yellow fever risk area. Vaccination is generally not given, but can be considered after a detailed risk assessment where the benefit may outweigh the theoretical risk of foetal infection from the live virus; discuss with a specialist first. Revaccinate after pregnancy if risk continues. • Breastfeeding an infant under 9 months: there is evidence of transmission of live vaccine virus in breast milk to infants under 2 months. Seek specialist advice. • Living with HIV: there is limited evidence that the vaccine may

	be given safely where the CD4 count is above 200 with a suppressed viral load, and the antibody response may be reduced. Seek specialist advice. • Low dose corticosteroid or non-biological oral immune modulating therapy: long term low dose therapy is not usually considered sufficiently immunosuppressive and these patients can generally receive live vaccines. Data are limited, so specialist advice may be sought. • Record any specialist advice received, including who gave it and when, in the consultation record.
Actions if patient is excluded or declines treatment	Explain why the vaccine cannot be given and what the alternatives are. Where a certificate is required for entry and vaccination is contraindicated, offer a Medical Letter of Exemption and explain that acceptance is at the discretion of the destination country. Reinforce mosquito bite avoidance, which is the only other protection against yellow fever. Advise on alternative provision, including their GP practice or a specialist travel clinic, and document the advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Stamaril, yellow fever vaccine (live attenuated 17D-204 strain), powder and solvent for suspension for injection in a pre-filled syringe. One 0.5 ml dose after reconstitution.
Legal category	POM
Storage	Store at +2°C to +8°C. Do not freeze. Store in the original packaging to protect from light. Reconstitute with the supplied solvent and use immediately; discard any unused reconstituted vaccine. Vaccine exposed to conditions outside the stated range must be quarantined and risk assessed in accordance with UKHSA Vaccine Incident Guidance.
Route/method of administration	• Reconstitute with the supplied solvent, mix gently, and administer immediately. • Give 0.5 ml by subcutaneous injection, usually into the upper arm. • Inspect visually before administration and do not use if the appearance differs from that described in the SPC. • Where given with other vaccines, use separate sites, preferably different limbs, or at least 2.5 cm apart, and record the site of each.
Dose and frequency of administration	A single 0.5 ml dose. The certificate becomes valid 10 days after a first dose and then lasts for the life of the person vaccinated, so routine revaccination is not required. Where a patient was vaccinated during pregnancy, or is immunosuppressed, revaccination may be considered on specialist advice.
Quantity to be administered and/or supplied	One 0.5 ml dose.
Maximum or minimum treatment period	Single dose. Vaccinate at least 10 days before travel where a certificate is required.
Disposal	Dispose of used syringes, needles and any reconstituted vaccine in a UN-approved puncture-resistant sharps container in accordance with local arrangements and HTM 07-01.
Drug interactions	May be given at the same time as inactivated vaccines at a separate site. Where another live vaccine is indicated and has not been given on the same day, separate the two by at least 4 weeks. The immune response may be reduced in individuals on immune modulating therapy.
Adverse effects	Very common and common: injection site pain, redness and swelling,

	headache, malaise, myalgia and fever. Rare: hypersensitivity reactions including anaphylaxis. Very rare but serious: yellow fever vaccine associated neurologic disease (YEL-AND) and viscerotropic disease (YEL-AVD), which occur almost exclusively after a first dose and more often in those aged 60 and over. Refer to the current SPC for the full list.
Yellow card reporting	Report suspected adverse reactions to the MHRA via the Yellow Card scheme at https://yellowcard.mhra.gov.uk . Any suspected YEL-AND or YEL-AVD must be reported and the patient referred urgently. Document any adverse reaction and inform the GP.
Records to be kept	Record the following information: • that valid informed consent was given by the individual, or by a person with parental responsibility, or that a decision was made in the individual's best interests in accordance with the Mental Capacity Act 2005 • name of individual, address, date of birth and GP with whom the individual is registered • name of immuniser • name and brand of vaccine • date of administration • dose, form and route of administration • quantity administered • batch number and expiry date • anatomical site of vaccination • advice given, including advice given if the individual is excluded or declines immunisation • details of any adverse drug reactions and actions taken • that the vaccine was administered via PGD • batch number, expiry date and anatomical site of administration • the certificate number issued and the date from which it is valid • any specialist advice obtained, including who gave it and when Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. The individual's GP should be informed. • Adults aged 18 years and over: keep records for 8 years • Children aged under 18 years: keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Offer the marketing authorisation holder's patient information leaflet, and issue the International Certificate of Vaccination or Prophylaxis completed and signed in accordance with NaTHNaC requirements. Explain that the certificate becomes valid 10 days after this dose and then lasts for life, and that a replacement can be obtained if lost.
Follow-up advice to be given to patient or carer	Advise on possible adverse effects and their management, and to seek urgent medical attention for fever, jaundice or severe illness in the days and weeks after vaccination, mentioning that they have recently received yellow fever vaccine. Reinforce mosquito bite avoidance, which also protects against dengue, Zika, chikungunya and malaria, none of which this vaccine covers. Advise that malaria prophylaxis may still be required for the destination.

Key references

- Immunisation Against Infectious Disease (the Green Book), Chapter 35 (Yellow fever)
- NaTHNaC: Contraindications and precautions for administering yellow fever vaccine, updated 27 May 2026
- NaTHNaC Yellow Fever Zone: conditions of designation and code of practice for Yellow Fever Vaccination Centres
- Summary of Product Characteristics for Stamaril, Sanofi, current version
- MHRA Drug Safety Update: yellow fever vaccine (Stamaril) and fatal adverse reactions, extreme caution in people who may be immunosuppressed and those 60 years and older
- NaTHNaC / TravelHealthPro country-specific recommendations
- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates	

Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or vaccine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC:		14/08/2026

	6047293		
Chris Pilkington	Pharmacist GPhC: 2046322		14/08/2026

Change history

Version number	Change details	Date
001	Development and issue of new PGD	30th July 2026

VACCINE SAFETY REQUIREMENTS, ADDED AT VERSION V002, 9 September 2026

The requirements on the following pages form part of this Patient Group Direction and must be met before any vaccine is administered under it. They were absent from the signed document and were added by this reissue after an estate-wide safety sweep of every vaccination PGD.

This reissue adds the safety requirements below and changes nothing else. It is NOT a clinical review of this PGD, and one remains outstanding.

Observation after vaccination

OBSERVE EVERY PATIENT FOR 15 MINUTES AFTER VACCINATION, SEATED.

Anxiety-related reactions including vasovagal syncope, hyperventilation and transient visual disturbance or paraesthesia can occur before or after any injection, and are commonest in adolescents. They are a response to the needle, not to the vaccine.

Have procedures in place to prevent injury from a faint.

Record that the observation period was completed.

Version and change record

Version: v002

Issued: 9 September 2026

Supersedes: the previous signed version of this document.

Change: the vaccine safety requirements above were added following an estate-wide sweep of every vaccination PGD, which found that the signed documents were missing, between them, any requirement for adrenaline to be available, any anaphylaxis provision, a stated observation period, a cold chain excursion procedure, a sharps disposal provision, a batch number requirement, and any wording on consent in children and young people. The specific items added to THIS document are: Observation after vaccination.

Authorised by Nitin Shori, Medical Director (GMC 6047293) and Chris Pilkington, Head Pharmacist (GPhC 2046322), on 9 September 2026. Signatures applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.

Signed on behalf of Get Real Health

Version v002, 9 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		9 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		9 September 2026

Both authorising signatories reviewed this version together on 9 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.